

Viatis Inc VTRS

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by

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			2026	2027
Price:	12.84	EPS	2.47	2.70
Shares Out. (in M):	1,152	P/E	5.2	4.8
Market Cap (in \$M):	14,800	P/FCF	0	0
Net Debt (in \$M):	12,600	EBIT	0	0
TEV (in \$M):	27,400	TEV/EBIT	0	0

Description

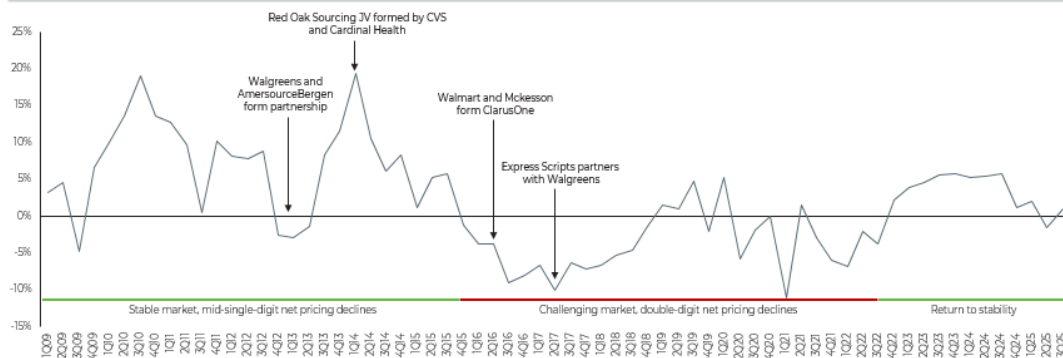
Summary

- Viatis (VTRS, \$14.8bn market cap) is one of the largest generic drug manufacturers in the world and is globally diversified across 165 countries.
- The company has a stable base business growing constant currency topline at 1-2% and should generate low double digit organic EPS growth over the next three years from organic growth, cost cuts and buybacks. The company has a policy of returning 50% of free cash flow to investors via dividends and buybacks and will use the remaining 50% on business development. The company also has \$815mm in proceeds from the recent sale of its Biocon Biologics stake to further allocate towards value enhancing actions.
- We believe consistent operational performance and pipeline catalysts in 2026 can drive a re-rating. We are particularly excited by clinical trial readouts later this year in Selatogrel and Cenerimod that could be game changers.
- The stock currently trades at 5.2x 2026e consensus EPS and is cheap without any pipeline success. With no pipeline success we think there is 100-150% upside, and with pipeline success we think the upside is 140-230%.

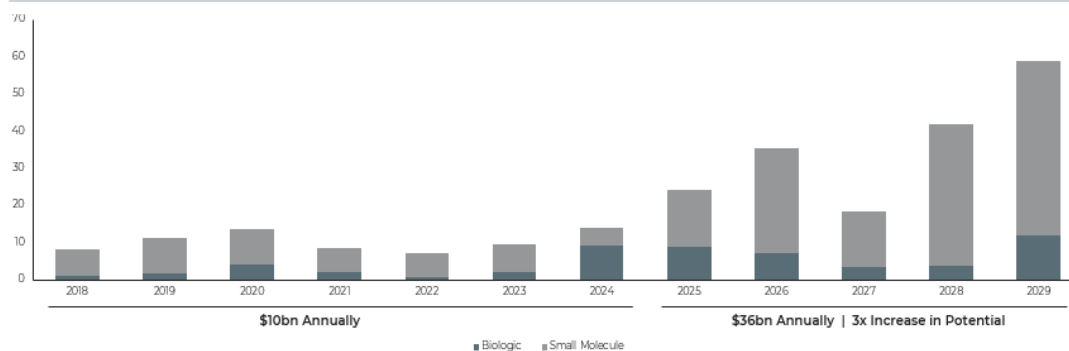
Industry update

- We discussed VTRS and the outlook for the generic drug industry previously in our Teva writeup in January 2024 and now believe that Viatis deserves its own writeup. Since 2024, we've seen continued evidence of stabilization in the US generics industry with Iqvia wholesale pricing trends remaining stable. In addition, there is a significant patent cliff of drugs with loss of exclusivity ("LOEs") that will go off patent over the next few years and be eligible for generic competition. This represents new sources of revenue for the generic industry.

AVERAGE US QUARTERLY WHOLESALE PRICE GROWTH



BRANDED REVENUE LOSING EXCLUSIVITY BY YEAR (\$BN)



Viatis company background

- The company was formed in 2020 through the merger of Mylan (a generic drug manufacturer) and Pfizer's Upjohn unit (consisting of Pfizer's global sales of branded products that have lost exclusivity). The business is globally diversified and operates in 165 countries, with 23% of revenue from North America, 37% from Europe, 16% from China, and 23% from other emerging markets and Asia ex-China.
- At the time of the Upjohn merger, management described a two phase journey post- merger. Phase 1, which Viatis completed in mid-

2024, consisted of divesting non-core assets to delever the balance sheet through asset sales and position the company for capital returns and growth. These non-core asset sales resulted in \$7bn of proceeds at an average multiple of 12.4x EBITDA. Phase 2, which Viatris is in now, focuses on sustainable long-term growth via organic and inorganic investments, as well as a commitment to return 50% of FCF to shareholders.

- The diversity across products and geographies has resulted in stable operational performance, with 7 quarters of divestiture and fx adjusted revenue growth from Q2 2023 to Q4 2024. At the end of 2024, VTRS was positioned with a clean balance sheet and a stable path to high single digits earnings growth.
- In December 2024, the company received a warning letter and import alert from the FDA for its facility in Indore, India. This resulted in a complete ban of US imports from this facility except for a few limited source products. This import ban resulted in a negative \$385mm EBITDA impact in 2025. \$50mm of that impact is one time failure-to-supply penalties which won't be repeated in 2026. We don't assume any incremental benefit from Indore in 2026 beyond the reversal of these penalties.
- Viatris has begun an enterprise-wide review that should result in significant cost savings over the next three years. While the company hasn't quantified the exact magnitude of the program, we conservatively estimate that the company can generate \$500mm of cost cuts over three years, of which \$200mm will be reinvested in R&D, resulting in \$300mm of net savings. The net cost take-out represents ~3% of the total company cost base while allowing the company to increase its current R&D budget by ~20%. This increase in R&D should continue to drive returns (and multiple expansion) as it invests in the pipeline and accelerates growth.
- Looking forward, we expect Viatris to continue to grow organic topline in 2026. This year presents a large opportunity for new generic launches given the large dollar amount of LOEs. Indore should become a small tailwind instead of a headwind. 2026 should be the first year since Mylan and Upjohn merged that headline revenue and EPS will grow. We believe Viatris will re-rate as the consistent organic performance comes through on headline numbers. We expect the company to continue to allocate capital to capital return and accretive tuck-in BD deals.

Pipeline Optionality

- On top of the consistent organic opportunity, Viatris has several pipeline opportunities which are not reflected in the share price at all.
- In February 2024 Viatris in-licensed two assets from Idorsia that were in the middle of their phase 3 trials: Cenerimod for systemic lupus erythematosus ("SLE") and Selatogrel for acute myocardial infarctions ("AMI", aka heart attacks). Both assets have been described by Viatris management as having blockbuster potential.
- We believe VTRS got these assets at a compelling price due to the stressed nature of the seller. Idorsia is a Swiss biotech company that was experiencing a significant cash crunch as it was launching a commercial product at the same time it was running these two phase 3 trials. Viatris acquired worldwide rights to the two assets for \$350mm upfront along with milestones and tiered royalties from mid-single digits to low-double digits. Idorsia agreed to fund an incremental \$200mm of development costs. Due to incremental liquidity challenges, the deal was further restructured in February 2025, reducing Idorsia's co-development funding obligation by \$100mm in exchange for a \$250mm reduction in milestones.
- Cenerimod is a daily oral s1p1 receptor modulator for moderate to severe SLE. SLE is a very heterogenous autoimmune disease with significant unmet need. Currently two biologic drugs are approved for SLE: Benlysta and Saphnelo. Consensus expects peak sales of \$2bn and \$1.4bn for Benlysta and Saphnelo respectively. Despite these high expectations, both drugs have a high non-responder rate (~40-50%). While Cenerimod has strong scientific rationale, we consider Cenerimod a higher-risk opportunity. The phase 2 trial missed its primary endpoint. Idorsia moved the asset into phase 3 after conducting a post hoc analysis on patients with high IFN-1 gene expression signature. In general, trials based on a post-hoc analysis have a lower probability of success even when there is strong scientific rationale. Cenerimod's phase 3 trial will likely read out towards the end of 2026.
- Selatogrel is a self-administered subcutaneous injection for patients experiencing the symptoms of a heart attack. Conceptually, time is muscle during a heart attack. Every minute an artery stays blocked, more heart muscle dies and doesn't come back. Selatogrel works by inhibiting platelet aggregation. It stops the clot from growing and may help dissolve a fresh clot before it has time to harden. Selatogrel achieves therapeutic levels in ~15 minutes, preventing significant heart damage between the time of the heart attack and the time the patient reaches the emergency room.
- Selatogrel had strong phase 2 data across 345 patients with 89% achieving the primary endpoint. The drug was well tolerated with no major bleeding events. The phase 3 trial is an outcomes based trial, and is expected to enroll 14,000 patients. The phase 2 data strongly indicates the drug works. Beyond the general risks of a phase 3 clinical trial, which admittedly are higher for an outcomes-based cardiovascular trial, there are two key risks we are focused on. First, can patients appropriately recognize heart attack symptoms and inject Selatogrel at the right time? Second, will we see any severe adverse bleeding events in the phase 3 trial? With regard to the first risk, the company has enrolled 10,000 patients so far. According to VTRS, it has received consistent feedback from principal investigators that patients are recognizing symptoms and injecting at the right time. For the second risk, the company has already been through 12 meetings with the data monitoring committee of the trial which has continued to allow the trial to progress. While the company hasn't disclosed the exact safety thresholds, it has described the safety criteria as very tight. Under Idorsia ownership, the trial was significantly underfunded. Since VTRS has taken control of the trial, it has significantly expanded the number of enrollment sites. As a result, the phase 3 trial will likely read out towards the end of 2026.
- We think of Selatogrel as "epipen for heart attacks". Currently patients receive an oral p2y12 inhibitor in the ambulance or emergency room, and then they take several hours to be absorbed. Selatogrel acts like a bridge. The ideal time to prescribe selatogrel to a patient is when they leave the hospital after their first heart attack. Cardiologists are very conservative doctors, so commercialization of Selatogrel will require effort to change doctor's practices. With 800,000 heart attacks per year in the US, there is significant commercial opportunity for Selatogrel.
- In addition to readouts on the Idorsia assets, Viatris has several other smaller catalysts in 2026 including approval for Effexor for general anxiety disorder in Japan in H1 2026, approval of fast acting meloxicam for acute pain in H2 2026, approval of low dose estrogen patch for contraception in H2 2026, approval of phentolamine ophthalmic solution for presbyopia in H2 2026, and others.

Valuation

- We think VTRS is substantially undervalued even without the pipeline.
- VTRS has gone through a significant transformation over the last 4 years. Until this year, consistent organic top-line growth has been masked by divestitures, FX headwinds, and the Indore import ban.
- The rest of the generic industry has seen a significant re-rating over the past two years. On 1y forward EBITDA multiples, Teva has re-rated from 6.2x to 10.3x, Amneal has re-rated from 7.5x to 9.7x, and Sandoz has re-rated from 5.9x to 16.1x. Meanwhile VTRS has only re-rated from 6.2x to 6.8x.
- Over the next three years, we think VTRS can grow EPS at a low double digit CAGR from a stable base business with 1-2% organic topline growth,

cost cuts, and consistent capital return.

- We think VTRS can trade at 8-10x our 2028e estimated EPS of \$3.20, with another \$5-10 per share of pipeline value, representing 100-150% upside just on the core business and 140-230% upside including the pipeline.

VTRS Valuation (\$/share)		
	Base	Upside
2028e EPS	3.20	3.20
Multiple	8.0	10.0
Core Value	25.6	32.0
% Return	99%	149%
Pipeline Value	5.0	10.0
Value Including Pipeline	30.5	42.0
% Return	138%	227%

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Catalyst

- Inflection to headline earnings growth
- Share buybacks
- Pipeline readouts

Messages

Subject Cap Alloc
Entry 01/17/2026 04:41 AM
Member virtualodin
 You wrote -

The company has a policy of returning 50% of free cash flow to investors via dividends and buybacks and will use the remaining 50% on business development.

What do you think "business development" actually means? 50% of FCF is a decent chunk of \$\$s and the hurdle for those type of investments should probably be v high if the stock is as cheap as your write-up suggests, any ideas what type/size of assets they'd be looking for? Do you have confidence in them creating value over time as they do this?

Also slightly odd to see a company with net debt ~~~ market cap not talking about allocating any FCF to reducing leverage? Do you think that stance makes sense? If we're to get a proper re-rating here, a healthier balance sheet won't hurt?

Ty!

Subject what went wrong?
Entry 01/17/2026 08:00 PM
Member dannny

besides the indore issue, whats the issue with the re-rate vs comps? is it just no branded exposure? also curious what the re-rate wouldve been with pro forma numbers

also on indore, what is the tailwind you mentioned (besides addback)? is it resumption? what's the chance of it in 2026 (looks like VTRS has been dragging their feet on this one)

thanks!

Subject Re: Cap Alloc
Entry 01/20/2026 08:24 AM
Member aviclara181

Regarding business development: the company has consistently messaged that they are targeting accretive commercial stage branded products that can enhance the company's long term growth profile. While the company hasn't specified a specific hurdle rate for BD, they've said that BD must compete with buybacks. We believe recent announced deals such as the acquisition of Aculys Pharma or the licensing of sotagliflozin from Lexicon Pharmaceuticals have a strong strategic rationale and are very accretive. These assets are commercial stage and leverage the company's existing commercial footprint.

Regarding the debt: The company is currently investment grade rated by Moody's (Baa3) and Fitch (BBB), and rated BB+ by S&P. We estimate the company will end 2025 with 3.0x Net debt/ebitda. Given the non-cyclical nature of the business, we view the leverage profile as reasonable and doesn't require further debt reduction.

Subject Re: what went wrong?
Entry 01/20/2026 08:25 AM
Member aviclara181

We believe the re-rating is primarily due to the fact that the comps have seen EBITDA growth, while VTRS has seen EBITDA shrink. Branded exposure is an input as well, as Teva has seen multiple expansion as their on market branded portfolio has grown and their pipeline readouts have been positive. The only tailwind from Indore is the elimination of failure to supply penalties in 2026. The company has been moving production to other facilities or has sourced third party supply for products produced at this facility. Whether or not Indore successfully comes back online will not impact 2026 numbers. We don't agree that VTRS has been dragging their feet. The remediation timeline is really driven by the FDA's re-inspection schedule which the company doesn't control.

Subject Any thoughts on the investor day?
Entry 03/28/2026 04:42 PM
Member Rtg123

Revenue growth looked in line with your thinking and above consensus but the company isn't guiding to much margin expansion and slightly slower EPS growth.